

USRad Repositioning – Implementation Plan v2

****August 3, 2026 | Internal Document | Not for Distribution****

Supersedes the six-phase plan. Restructured to three releases per advisor review.

****Governing constraint:**** change only what is false, contradictory, or blocking. Everything not listed here stays as written.

GOVERNING THESIS

> ****USRad is a prefunded imaging network. Some assignments are prepaid by the patient. Some are prefunded by an employer. Either way, funding assurance is secured before the assignment is routed.****

Four sentences to lock before any file is touched:

1. USRad is a prefunded imaging network.
2. Funding assurance precedes routing. Invoicing in arrears requires a documented exception.
3. One contracted rate per center per modality across funded lanes. Rates vary by market.
4. Cash-pay remains a supported lane. Self-funded employers become the primary commercial target.

No further strategic discussion is required before Release 1.

DECISIONS

****Blocking – settle now:****

Decision	
D1	Funding assurance before routing is the standard. Arrears invoicing is an exception requiring approval
D2	One contracted rate per center per modality across funded lanes; rates vary by market
D5	Cash-pay is a lane, not the company's identity

****Deferrable – do not block Release 1:****

Status	
D3	– partial member cost share Interim site language: <i>"Member cost-sharing, if any, is established by the employer program and collected by USRad – not the provider."</i> Settle before the employer agreement
D4	– workers' comp Already deferred. Only open question is how aggressively to remove it. Answer: de-emphasize, don't dismantle

RELEASE 1 – CORRECT WHAT IS FALSE

****Purpose:**** nothing public contradicts the prefunded thesis. No layout changes, no architecture changes.

1A – Provider page: prepaid vocabulary

File:line	Current	Change
`provider/HeroSection.astro:61`	We bring pre-paid patients.	pre-funded assignments
`provider/ScannerUtilization.astro:418`	Pre-screened. Pre-paid.	Pre-screened. Pre-funded.
`provider/GuaranteeSection.astro:74`	Pre-qualified patients who arrive.	Disambiguate – clinical or financial
`provider/FAQSection.astro:369-371`	patients pre-arrange their own payment	funding secured before appointment confirmed
`provider/FAQSection.astro:379-381`	because patients have pre-arranged payment	same substitution

`HeroSection.astro:61` is the highest-risk sentence on either page. False under every model, including prefunded – the employer paid, the patient did not. Three claims in the same sentence inherit the fix.

1B – Provider page: FAQ 6 rewrite

`provider/FAQSection.astro:312-333`. Self-contained `<details>` body. Currently defines USRad as cash-pay-only, asserts patient-as-payer, and promises no-claims workflow, all in a flat "No." to a compliance question.

Rewrite whole. Must hold under employer volume: prefunded network, two funding lanes, no claims under either, no overlap with the center's existing payer contracts.

1C – Employer page: remove the four claims-workflow statements

File:line	Action
`employer/DualSolution.astro:93-94`	"Imaging centers bill the program directly" – delete. Contradicts four /provider promises
`employer/ExecutiveFAQ.astro:224`	"Direct billing options for both WC and health claims" – delete
`employer/Implementation.astro:86`	"Claims workflow setup" – delete. Timeline holds at three bullets
`employer/IntegrationPartners.astro:63`	Remove "claims integration" from the three-item list

****Preserve:**** `Implementation.astro:51` (Claims data analysis) and `ExecutiveFAQ.astro:230` (Claims data feeds). Analytics, not funding mechanics. Fully compatible with prefunding.

1D – Compliance and performance claims

****Promoted into Release 1.**** Not caused by the repositioning, more urgent than most of it.

File:line	Claim	Issue
`employer/ExecutiveFAQ.astro:278`	licensed in all 50 states	No license type named
`employer/ExecutiveFAQ.astro:276`	HIPAA certified	No such certification exists
`employer/ExecutiveFAQ.astro:274`	ERISA compliant	Legal conclusion asserted flatly
`employer/ExecutiveFAQ.astro:281`	Workers' comp approved	Approval claim for a deferred product
`employer/ExecutiveFAQ.astro:185-186`	Average client saves \$1.7M	No live employer clients exist
`employer/ExecutiveFAQ.astro:76-77`	implemented this exact model 1,200+ times	Center-recruitment count reused as implementation count
`employer/Implementation.astro:131-132`	Our Implementation Never Fails / 168,000+	Absolute claim; third reuse of an AnciCare number
`employer/CostAnalysis.astro:97`	"Real Example: Lower Back Pain"	Undisclosed illustrative content. `EmployerCaseExample` models correct disclosure four times

1E – Workers' comp availability claims only

De-emphasize. Do not dismantle architecture.

- `employer.astro:22` – `` leads with Workers' Comp
- `employer/ExecutiveFAQ.astro:281` – covered in 1D
- `employer/ExecutiveFAQ.astro:175-176` – Month 6 WC disability reference
- `employer/Implementation.astro:107` – "HR/adjuster training"
- `employer/FinalCTA.astro:27` – "One solution for both populations"

</div>
<div data-bbox="15 591 983 631" data-label="Text">
<p>**Interim treatment for the WC architecture:** label it <i>"Future program expansion: workers' compensation and other payor-sponsored imaging programs."</i> Preserves `DualSolution`'s two-column layout, the calculator's WC input, and `IntegrationPartners` without representing any of it as launched.</p>
</div>
<div data-bbox="15 641 248 655" data-label="Section-Header">
<h2>## 1F – ProviderBrief PDF</h2>
</div>
<div data-bbox="15 665 801 681" data-label="Text">
<p>`provider/ProviderBrief.astro:186` → `/downloads/USRad\_Provider\_Opportunity\_Brief.pdf`</p>
</div>
<div data-bbox="15 690 968 718" data-label="Text">
<p>Un-versioned, gated, explicitly marketed for internal forwarding (`:174-180`). Every other artifact can be changed silently. This one is already in circulation.</p>
</div>
<div data-bbox="15 727 811 743" data-label="Text">
<p>Review against the prefunded thesis, version the file, republish. Separate deliverable.</p>
</div>
<div data-bbox="15 752 959 780" data-label="Text">
<p>**Release 1 outcome:** no provider is told every patient personally prepays. No employer is told USRad administers claims. No unsupported compliance claim remains. No layout changed.</p>
</div>
<div data-bbox="16 792 48 803" data-label="Text">
<p>---</p>
</div>
<div data-bbox="15 813 421 829" data-label="Section-Header">
<h2># RELEASE 2 – STATE THE EMPLOYER PROPOSITION</h2>
</div>
<div data-bbox="15 839 330 854" data-label="Text">
<p>**Three new ideas. Nothing more.**</p>
</div>
<div data-bbox="15 863 968 929" data-label="List-Group">

1. **The employer is the buyer**, not the employee.
2. **\$0 member cost is the steering mechanism**, not a benevolence message. Anything the employee pays leaves the hospital a live option.
3. **Funded account with monthly reconciliation** The employer maintains an agreed funding arrangement before routing and receives a consolidated statement.

</div>
<div data-bbox="15 937 313 954" data-label="Section-Header">
<h2>## 2A – Hero proposition rebuild</h2>
</div>
<div data-bbox="15 967 986 983" data-label="Page-Footer">
<p>//home/usrad/Web Development/usrad-marketing/docs/workstream-a/source/2026-08-03-repositioning-plan-v2.md</p>
</div>

`employer/EmployerHero.astro:68` – "\$260 MRIs (not \$3,200)"

****Budget this as thinking, not typing.**** Small in file count, but it is the page's core proposition. Current framing sells employee affordability; the replacement sells employer spend reduction with \$0 as the steering lever.

The supporting arithmetic already exists on the page: imaging currently landing on the plan at hospital rates, plus `IndustryData.astro:33` (67% skip, EBRI 2023), `:99` (+\$847 per delayed diagnosis), `:79` (31% longer disability). Skipped imaging is deferral, not savings.

2B – Member-affordability examples

- `employer/DualSolution.astro:175` – "\$260 MRIs employees can actually afford"
- `employer/CostAnalysis.astro:180` – "Day 1: Pain starts, books \$260 MRI"

2C – Funding language insertion

****Primary:**** `employer/Implementation.astro:47-53`, Week 1. `:50 • Execute master agreement` is the money moment and says nothing about money. Plain four-item `

`; a fifth bullet drops in at zero layout cost.

****Secondary:**** `employer/ExecutiveFAQ.astro` – new `` between Q3 (closes `:190`) and Q4 (opens `:192`). Five executive Q&As and none asks how the employer pays. Uniform accordion stack; a sixth entry is purely additive.

****Do not use:**** `ROI Calculator.astro:157-165`. Sits inside an animated results grid driven by `animateValue` at `:604-611`.

2D – Rate labeling

Market-based pricing is the model, not a defect. The prices are unlabeled, not wrong.

File:line	Add context
---	---
`employer/EmployerHero.astro:68`	\$260
`employer/ROI Calculator.astro:123`	\$350
`employer/EmployerCaseExample.astro:204`	\$420
`employer/ROI StatPanel.astro:49`	\$640

`ROI StatPanel:69` hands off to `ROI Calculator` and they render back to back at `employer.astro:42-44`. Those two need consistent framing at minimum.

Two genuine conflicts:

- `employer/DualSolution.astro:110` – "Transparent ****flat**** rates" → fixed per market
- `employer/DualSolution.astro:287` – "Combined volume drives even better pricing" – volume cannot move price under D2

****No longer requires change**** (reversing prior guidance): `MarketScopeShowcase` strategy panel, `FoundingPartners.astro:211-215`, `HowItWorks.astro:133`, `ConsultationCTA.astro:89`. Market-varying rates are the model.

RELEASE 3 – PSA AND MINIMUM FUNDING GATE

3A – The narrowed PSA approach

****Do not draft a broad v7.0.**** The advisor's question – can the existing Fully Funded architecture be generalized without introducing a second provider workflow – resolves yes.

****Broaden, do not rename.**** v6.1 § 1.7 already reads: *"Fully Funded Assignment" means an Assignment for which USRad has secured funding prior to the provision of services.* That definition already covers employer-prefunded work; nothing in it names the patient as payer.

Keeping the term avoids:

- the `assignment\_type` enum migration (`fully\_funded` remains valid)
- `renderAssignmentTypeBadge()` changes
- a migration addendum for providers already executed under v6.1

The term is cosmetically awkward once Claims-Funded goes dormant. Not worth a platform migration.

****The routing warranty should be deleted, not reworded.**** The advisor correctly caught that the proposed phrasing implied USRad owes *only* where the gate failed. But their three-part fix is redundant: if payment is unconditional on fulfillment, no carve-out is needed.

v6.1 § 4.3 already is that promise: *"For Fully Funded Assignments, USRad shall disburse Provider compensation within ten (10) business days following Fulfillment Complete."* No funding condition. Unconditional as written.

The funding gate stays as ****internal routing policy****. Putting it in the PSA would create a provider breach claim for something that causes the provider no harm.

****Net PSA scope:****

Action	Item
--- ---	
Broaden	§ 1.7 – clarify funding may be secured from the member, employer, sponsor, or other approved source
Preserve	§ 4.3 payment trigger, unchanged
Deactivate	Dormant Claims-Funded requirements that do not apply
Fix	The v6.1 defect list below
Add	Anti-circumvention; BAA mechanism

****No Article IV structural change. No new Assignment Types.****

3B – v6.1 defects (independent of the repositioning)

Defect	Location
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§ 4.1 "regardless of Assignment type" contradicts Exhibit A "exclusively to Fully Funded"	Article IV / Exhibit A
§ 4.5 Member Non-Collection absent from survivability	§ 9.1
§ 2.7 requires electronic claims for work generating no claim	§ 2.7
No BAA requirement anywhere	Article II
No anti-circumvention provision	Article VI or new
Exhibit A "Program Expansion" overrides Article VIII negative consent	Exhibit A

Anti-circumvention becomes urgent the moment USRad introduces a center to an employer.

3C – Minimum Funding Gate

****Operational definition, required before engineering, operations, counsel, and sales can use the term consistently:****

> ****Funding assurance**** means an active and approved funding arrangement sufficient to support routing under USRad policy. It may be satisfied by secured patient payment, an employer-funded account with sufficient available balance, assignment-specific prefunding, or another mechanism approved by USRad.

This belongs in the Founder Decision Memo and the Funding Gate specification. It does ****not**** belong in the provider PSA – the provider does not need to reason about how USRad is funded.

Specify only. Do not build the employer platform.

Seven checks before routing:

1. Employer program active
2. Member eligible
3. Valid physician order
4. Approved service
5. Funding arrangement active
6. Sufficient available balance or capacity
7. Routing permitted

Balance tracking, reconciliation statements, and employer dashboards follow once an employer implementation is real.

This gate is the load-bearing build. It is what makes the contract language and the site copy true.

EXPLICITLY OUT OF SCOPE

Cut from the immediate work order. May become real work later; none is necessary to make the current positioning truthful.

****Structural rebuilds:**** `DualSolution` two-column architecture · full WC removal · `ROI Calculator` restructuring · `AssignmentFlowDiagram` redesign · `ReferralWorkflow` grid arity · `GuaranteeSection` grid · `ScannerUtilization` two-lane card

****Platform:**** claims intake · 837 processing · repricing engine · TPA integration · treasury automation · employer-specific settlement variants · credit underwriting policy beyond "exceptions require approval"

****Documents:**** full Provider Manual · Payor Assignment Addendum · Funding Schedule Tiers 3 and 4 (arrear variants are exceptions under D1, so the schedule reduces to prefunded arrangements plus an exception process)

****Leave as written:**** `TrustBar` · `ProvenSuccess` · `ProviderSpineNav` · `NetworkBuilding` · `MarketOpportunityTeaser` · `CredibilityBar` · `IndustryData:27-86` sourced tiles · `EmployerCaseExample` disclosure apparatus · `ReferralWorkflow:318-325` · provider FAQs 1, 4, 5, 8, 9

****Dead code – do not touch:**** `PriorityMarketsMap` (imported at `provider.astro:17`, never rendered) · `provider/Test.astro` · `ExitModal.astro.backup` · `employer/HeroEmployer.astro` · `employer/PartnerMfgGrid.astro`

Note `PartnerMfgGrid` asserts manufacturer endorsement (Siemens/GE/Philips/Canon/Hitachi). Ships nothing today, live in the tree.

****Documented, not repaired:**** `ROI Calculator.astro:593` computes the return-to-work line as 16% of **total** savings including health-plan scans, then labels it "(WC)". Math defect with no mechanical relationship to the WC input. Real, but not part of this repositioning.

TRACK 4 – EMPLOYER CONTRACTING ARCHITECTURE

****Parallel track, not a fourth release.**** Established so it does not remain an indefinitely acknowledged gap while marketing and provider work proceed.

****The employer agreement does not exist.****

All three releases leave USRad with a truthful site and a clean PSA, and still nothing to sign an employer with. Required document set:

- Employer agreement (services, term, rates, data and PHI handling, ERISA positioning, indemnity, termination)
- Funding arrangement terms (reduced scope per D1 – prefunded plus exception process)
- Employer rate exhibit
- BAA

This is correct sequencing, since provider recruitment is the near-term goal and does not require it. But it is the item standing between USRad and revenue, and it should stay visible rather than sitting behind work that feels productive.

****Open question for counsel, independent of everything above:**** does the intended employer contracting and money-movement structure require a certificate of authority under Fla. Stat. § 626.88? Under prefunding with no member collection and no plan funds held, exposure is materially lower than under a claims model. Confirm before the employer agreement is drafted.

SEQUENCE

****This week:**** lock D1, D2, D5 in a one-page founder decision memo. No file changes before that.

****Release 1:**** 1F (PDF, already in circulation) → 1A and 1C (nine line-level edits) → 1D (compliance) → 1B (FAQ 6) → 1E (WC availability).

****Release 2:**** 2C first (cheapest, locks prefunding publicly) → 2A → 2B → 2D.

****Release 3:**** 3B (v6.1 defects, independent of everything) can start in parallel with Release 1. 3A and 3C after D1.

****Everything about billing mechanics and carrier positioning resolves on the employer page. Everything about company narrative resolves on the provider page. Rates resolve on both.**** Two edit passes, different owners, parallelizable once D1 is settled.

Prepared for advisor review. Companion documents: /provider audit, /employer audit, PSA v6.1→v7.0 Change Analysis, Position Revision – Funding Gate.